Cagrilintide Available for Research Use Only

(0.3 – 4.5 mg / week, continuous)

Cagrilintide is a long-acting amylin analogue developed for potent appetite suppression and weight loss; clinical phase-2 data show dose-dependent weight reduction versus placebo and active comparators, and extensive preclinical work maps its brainstem/hypothalamic satiety mechanisms.

💉💉 Probably safe (some human evidence)

⭐⭐ Probably effective (some positive human signals)

Disclaimer: These notes are not authoritative and should not be interpreted as definitive scientific guidance. They are summaries, compilations and plausible extrapolations drawn from the most relevant research I was able to locate, and may omit nuances, conflicting data, or emerging evidence. The material reflects an attempt to organize and understand complex topics, not to provide clinical recommendations or expert interpretation.

This document was updated 11-Aug-26. The most recent version of this document is available at https://pdfs.researcher6076.com/pdfs/cagrilintide.pdf

Cagrilintide has substantial human clinical evidence as both monotherapy and in combination with semaglutide. Phase 2 monotherapy studies demonstrated **dose-dependent weight loss** with once-weekly Cagrilintide, while subsequent Phase 3 CagriSema trials confirmed **substantial weight loss when cagrilintide 2.4 mg was combined with semaglutide 2.4 mg once weekly**.

In REDEFINE 1, adults with overweight or obesity without diabetes receiving CagriSema 2.4/2.4 mg once weekly had an estimated mean body-weight reduction of approximately 20% at 68 weeks. Cagrilintide 2.4 mg and semaglutide 2.4 mg monotherapy arms were also included, confirming **greater weight loss with the combination than with either component alone**.

Longer-term cardiovascular-outcome and rare-event safety characterization remains incomplete despite the availability of large Phase 3 safety datasets.

Dose

Cagrilintide is a **long-acting amylin analogue designed for once-weekly subcutaneous administration**. Its terminal elimination half-life is approximately **7–8 days**, supporting weekly rather than daily dosing.

Doses Studied as Monotherapy

The Phase 2 dose-ranging obesity trial evaluated:

- **0.3 mg once weekly**
- **0.6 mg once weekly**
- **1.2 mg once weekly**

- **2.4 mg once weekly**
- **4.5 mg once weekly**

Weight loss was dose-dependent, with progressively greater mean weight reduction across the dose range. The 4.5 mg dose produced the greatest weight reduction in the Phase 2 monotherapy study, but gastrointestinal adverse effects also increased with dose.

The clinical studies used **gradual dose escalation rather than beginning immediately at the target dose**, primarily to improve gastrointestinal tolerability.

Dose Used with Semaglutide

The principal CagriSema regimen studied in Phase 3 is:

- **Cagrilintide 2.4 mg + semaglutide 2.4 mg SC once weekly**

The components are escalated toward the target dose rather than initiated at full dose.

The 2.4 mg cagrilintide dose should therefore be regarded as the best-characterized current dose for combination therapy with semaglutide, while cagrilintide monotherapy has been studied at doses as high as 4.5 mg weekly.

Dose Escalation

Slow titration is important because gastrointestinal intolerance is dose-related. Nausea, vomiting, constipation, reduced appetite, and early satiety are the principal tolerability limitations.

The available trials support the general principle:

low starting exposure → gradual escalation → individually tolerated maintenance exposure

rather than rapid escalation to the maximum studied dose.

Specific titration schedules should be identified by trial when discussed because monotherapy and CagriSema studies have not all used identical escalation schemes.

Frequency

Once weekly is the appropriate research-based dosing frequency.

Because cagrilintide has an approximately 7–8-day terminal half-life, there is no established pharmacologic rationale for routine daily administration. Likewise, splitting a weekly dose into multiple injections per week has not been established as superior to once-weekly administration in the major clinical trials.

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Dose Summary

Benefits

- **Dose-dependent weight loss** — demonstrated in human monotherapy trials.
- **Marked appetite suppression and increased satiety** — supported by human and mechanistic data.
- **Reduced meal size** — consistent with amylin-receptor pharmacology and experimental evidence.
- **Greater weight loss when combined with semaglutide** — demonstrated in Phase 2 and Phase 3 trials.
- **Complementary amylin/GLP-1 appetite regulation** — strong mechanistic rationale.
- **Improved glycemic parameters with CagriSema** — demonstrated particularly in subjects with impaired glucose regulation or type 2 diabetes.

Indications

- **Obesity or overweight with comorbidities** — *Human clinical trials.*
- **Adjunct to GLP-1 therapy for plateaued weight loss** — *Consistent individual / community reports; mechanistic rationale.*
- **Research use in metabolic physiology studies** — *Preclinical and translational research.*

Contraindications

- **Pregnancy and breastfeeding** — *Standard for weight-loss agents; theoretical fetal risk.*
- **Uncontrolled eating disorders** — *Clinical caution; may worsen disordered intake.*
- **Severe gastroparesis** — *Because of further gastric emptying delay; mechanistic concern.*
- **Unstable cardiovascular disease** — *Limited trial data; exercise clinical caution.*

Side effects

- **Nausea and transient vomiting** — *Human clinical data; dose-related.*
- **Early satiety and reduced caloric intake** — *Expected effect; can cause dizziness or weakness in some.*
- **Potential tachycardia or palpitations at higher doses (Rare)** — *Reported in some trials and animal studies.*
- **Possible hypoglycemia when combined with insulin or insulin secretagogues** — *Mechanistic risk; clinical vigilance required.*

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Biological mechanism

Cagrilintide is a long-acting amylin analogue that activates calcitonin receptor/receptor activity-modifying protein complexes forming amylin receptors, including AMY1, AMY2, and AMY3 receptors.

Its weight-loss effects appear to arise predominantly through central regulation of appetite, satiation, and meal termination. Important neural targets include the dorsal vagal complex and associated brainstem/hypothalamic circuits.

Experimental work indicates that AMY1 and AMY3 receptor signaling contributes importantly to cagrilintide-induced weight loss.

Cagrilintide and GLP-1 receptor agonists act through complementary neural and metabolic pathways. This provides a mechanistic basis for the greater weight reduction observed when cagrilintide is combined with semaglutide.

The peptide has an elimination half-life of approximately 7–8 days, permitting sustained receptor exposure with once-weekly administration.

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